

# TRAINING SUMMARY REPORT

AKWOS Rightsholders, Club Members, Staff  
Volunteers on Therapy, Referral of Psychological  
Mental Health Cases and Stress Management  
in Rwanda from 22 - 23 December 2025



27 December 2025

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# Executive Summary

Wihogora Psychosocial Centre in partnership with AKWOS delivered an intensive mental health and stress management training for 40 rights holders, club members, staff, and volunteers from Rubavu and Musanze districts in the Western and Northern Provinces respectively.

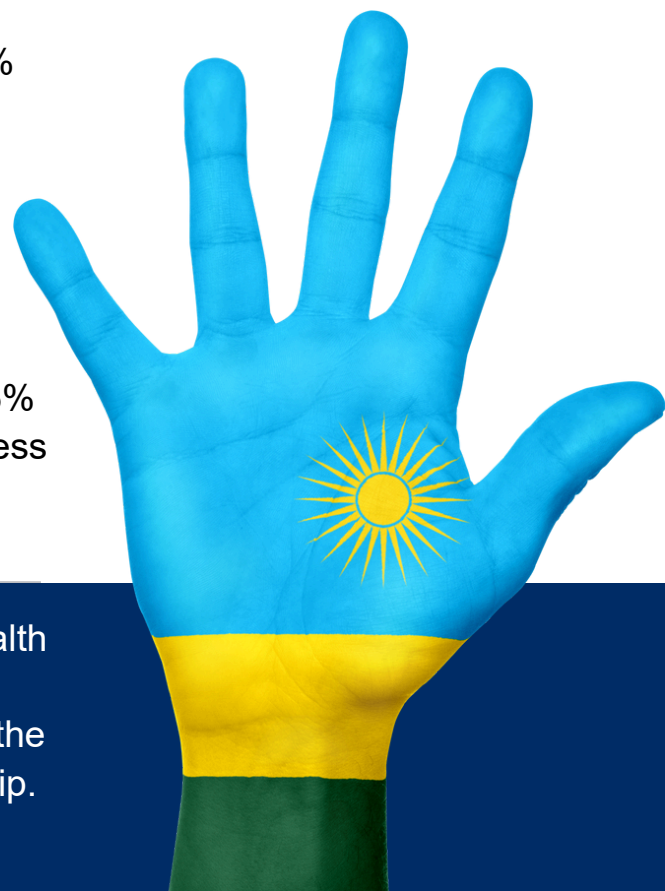
The training addressed the critical intersection of Gender-Based Violence (GBV), family conflict, and mental well-being in Rwanda.

By equipping community leaders and staff with psychological first aid and referral skills, AKWOS has strengthened its grassroots capacity to respond to trauma and promote resilience among vulnerable women and girls.

## Contextual Rationale and Mental Health Landscape

The training addressed a significant gap in healthcare access and awareness in Rwanda.

- **Prevalence vs. Treatment:** While 10.2% to 20% of Rwandans experience mental health conditions, far fewer receive professional treatment compared to those with physical illnesses.
- **PTSD and Depression:** Data from the Rwanda Mental Health Survey (2018) estimates that 25% of Rwandans struggle with Post Traumatic Stress Disorder (PTSD), and one in six suffer from depression.
- **Vulnerability of Active Generations:** Mental health issues are highest in those aged 26–35 (21%) and 46–55 (26.9%), categories that represent the core of the workforce and community leadership.
- **The Burden of History and GBV:** The 1994 Genocide against the Tutsi remains a major contributor to widespread disorders, further compounded by modern stressors such as extreme poverty, family conflict, and the multifaceted trauma of GBV.





# Training Objectives and Methodology



The primary goal was to move beyond theory and provide actionable tools for community-level support.

The core objectives were to:

- o Equip participants with basic knowledge of therapy practices and stress management.
- o Strengthen capacity to recognise signs of psychological distress in the community.
- o Formalise referral pathways for professional mental health services.
- o Promote self-care and resilience-building for AKWOS staff and volunteers.

## Methodology

The training utilised participatory methods, including brainstorming, small group discussions, and “MINGLE” relaxation exercises. Crucially, discussions were centred on real-life GBV scenarios to bridge the gap between training and daily community interactions.

# Key Content and Impact Areas



## A. Understanding the Spectrum of GBV and Trauma

Participants explored the various forms of violence prevalent at the domestic level, including physical, sexual, emotional/psychological, and economic violence.

A significant focus was placed on intergenerational trauma, helping participants understand how unsolved family conflicts and witnessed violence increase the risk of family violence in adulthood.

## B. Deconstructing Harmful Cultural Norms

The training facilitated a critical dialogue on the Kinyarwanda saying “**Niko Zubakwa**” meaning “that’s how marriages are built.”

Participants concluded that this belief is often used to silence victims of domestic and sexual violence. By identifying this as a barrier to health, participants are now better prepared to challenge harmful stereotypes in their communities.

## C. Recognising Signs of Distress

Participants were trained to identify the diverse symptoms of stress and mental health crises including:

- Physical: Chronic pain, fatigue, and heart palpitations.
- Mental/Behavioural: Lack of concentration, memory lapses, isolation, and substance abuse.
- Emotional: Bouts of depression, fits of rage, and loss of self-esteem.



# Value to AKWOS and Rights Holders

## Institutional Strengthening

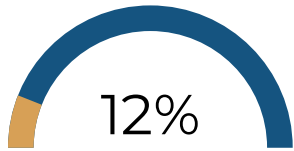
- **Crisis Management:** Staff and volunteers are now trained to intervene during emotional crises. During the training itself, the facilitators successfully supported two participants who experienced emotional crises during role-play sketches, demonstrating the necessity of these skills in the field.
- **Resilience:** By focusing on “Mental Health First Aid”, the staff is better equipped to manage their own job-related stress while serving traumatised populations.

## Empowerment of Rights Holders

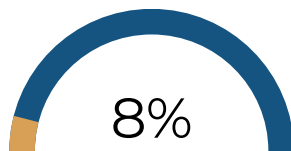
- **Safe Spaces:** The formation of trust-based groups (e.g., Ubumwe, Ibyishimo byacu) provided a safe environment for rights holders to share experiences of GBV and intergenerational trauma for the first time.
- **Actionable Pathways:** Rights holders now have a clear understanding of the GBV support pathways, including medical, security, psychosocial, and legal support.



# Evidence of Need



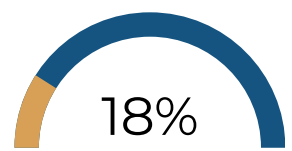
Rwandans suffer from severe depression



Rwandans suffer from PTSD

285

Recorded suicide cases in Rwanda by 2022 highlighting the urgency of mental health interventions



Of individuals who witnessed parental violence in childhood were later victims of sexual violence, illustrating the cycle of violence

NEED

The training results demonstrated a significant increase in mental health literacy and practical skills among the 40 participants. The following statistics and outcomes highlight the immediate impact on both rightsholders and staff:

## Quantitative Knowledge Improvement

A comparative analysis of pre-test and post-test results across 40 participants shows a clear quantitative leap in understanding:

- Initial Knowledge Baseline: The average score on the pre-test was 6.88 out of 10.
- Post-Training Achievement: Following the sessions, the average score rose to 9.1 out of 10, indicating a comprehensive grasp of the core concepts

## Critical Knowledge Gains:

- Culture and Awareness: Participant scores in understanding the intersection of culture and mental health improved from a total of 62 to 73 points.
- Defining Mental Health: Scores about the definition of mental health saw the largest jump, increasing from 181 to 252 total points.
- Protective Measures: Knowledge of how to protect and maintain mental well-being rose from 32 to 39 total points.

## Qualitative Behavioural Changes

- Challenging Harmful Norms: Participants collectively concluded that traditional cultural beliefs, such as the “Niko Zubakwa” mindset, are harmful and often fuel domestic and sexual violence by encouraging silence.
- Development of Trust Circles: Through the “MINGLE” exercises, three balanced groups (Ubumwe, Ibyishimo byacu, and icyerekezo) were formed.
- These groups established enough intimacy and trust for participants to openly share experiences of intergenerational trauma and personal GBV issues.
- Capacity for Crisis Response: The training’s immediate value was proven when two participants experienced emotional crises during role-play sessions. The presence of trained staff and the application of Psychological First Aid (PFA) allowed these individuals to be supported and successfully reintegrated into the group.
- Mastery of Referral Pathways: Rights holders and staff reported a significantly improved understanding of the medical, security, psychosocial, and legal support pathways available to survivors, moving beyond a general awareness to actionable knowledge.

**BEYOND THE  
SCORES**  
the training  
facilitated  
immediate  
shifts in  
participant  
perspectives  
and emotional  
readiness.

# Conclusion

This training has transformed 40 community members into active agents of mental health awareness.

By addressing the root causes of trauma and providing clear referral mechanisms, AKWOS has significantly enhanced the safety and well-being of its rights holders.

Ongoing investment in these psychosocial support structures is vital for the long-term rehabilitation and re-integration of GBV survivors.

